

CONSENSUS-BASED NEURODEVELOPMENTAL ASSESSMENT REPORT*Autism Spectrum Disorder Assessment Pathway***CLIENT**

Azhaan Mohammed

DATE OF BIRTH

15 June 2019 · 6y 10m

PARENTS

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ASSESSOR

Vishal Maharaj

REGISTRATION

Registered Psychologist · PSY0001579010

PRACTICE

Azure Mind

DATE OF ASSESSMENT

1 Apr 2026

DATE OF REPORT

1 May 2026

REFERRING PRACTITIONER

Dr Example

SCHOOL

Example Primary School

YEAR LEVEL

Prep

PRONOUNS

he/him

ASSESSMENT CONTEXT

This assessment was conducted as part of a consensus-based neurodevelopmental assessment pathway to explore the presence of Autism Spectrum Disorder (ASD) and associated developmental, behavioural, social-communication, sensory, and/or functional concerns.

The assessment process included clinical interview, behavioural observations obtained during assessment, developmental history review, and consideration of available collateral information and supporting documentation where applicable.

Assessment findings are intended to contribute to broader clinical understanding regarding the client's current presentation, functional profile, and support needs across relevant settings. The findings may assist in informing ongoing clinical, educational, behavioural, and/or supportive care planning where appropriate.

CONSENT AND USE OF REPORT

Informed consent for assessment, collateral information review, and report preparation was obtained prior to commencement of the assessment process.

This report reflects clinical observations, developmental history, behavioural presentation, collateral information, and assessment findings available at the time of assessment and should be interpreted within the broader developmental and clinical context of the client.

This report has been prepared for clinical, educational, and supportive care planning purposes only and is not intended to function as a standalone medico-legal or forensic opinion.

The information contained within this report is confidential and should only be distributed to individuals directly involved in the client's care, educational planning, or support pathway unless otherwise authorised or required by law.

PRESENTING CONCERNS

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BACKGROUND

Pregnancy and birth

—

Early development

—

Educational history

—

Emotional, behavioural and sensory

No specific concerns were identified in this domain at the time of assessment. Further enquiry recommended during paediatric review.

Caregiver Observations (M-CHAT)

Caregiver observations from the M-CHAT screening are summarised here for the collateral section heading check.

DSM-5-TR CRITERIA (A & B)

The following section considers the client's presentation in relation to DSM-5-TR diagnostic criteria for Autism Spectrum Disorder (ASD). Clinical interpretation was informed by developmental history, behavioural observations obtained during assessment, collateral information, and available supporting documentation.

Each criterion area was considered according to the degree to which available evidence supported the presence of clinically significant neurodevelopmental differences associated with Autism Spectrum Disorder.

Rating scale

- 0 Not Supported
- 1 Partially Supported / Emerging Features
- 2 Supported
- 3 Strongly Supported

A. Social Communication and Social Interaction

Persistent differences in social communication and social interaction across multiple contexts, currently and/or by developmental history, as reflected in the following areas:

A1. Social-Emotional Reciprocity

This criterion considers differences relating to reciprocal social interaction, conversational reciprocity, shared emotional engagement, initiation and response within social interaction, and the capacity to sustain socially reciprocal communication across settings.

A2. Non-Verbal Communicative Behaviours Used for Social Interaction

This criterion considers differences relating to non-verbal communication, including eye contact, facial expression, gesture use, body language, and integration of verbal and non-verbal communication during social interaction.

A3. Developing, Maintaining and Understanding Relationships

This criterion considers differences relating to peer relationships, social understanding, imaginative interaction, adjustment of behaviour across social contexts, and the capacity to develop and maintain age-appropriate social relationships.

B. Restricted and Repetitive Behaviour, Interests and Sensory Regulation

Restricted and repetitive patterns of behaviour, interests and sensory regulation, currently and/or by developmental history, as reflected in the following areas:

B1. Repetitive Motor Movements, Use of Objects or Speech

This criterion considers repetitive or stereotyped motor movements, repetitive speech patterns, repetitive play behaviours, echolalia, unusual phrasing, repetitive use of objects, and other repetitive behavioural patterns.

B2. Behavioural Rigidity, Sameness and Inflexibility

This criterion considers distress associated with change, behavioural rigidity, transition difficulties, inflexible thinking patterns, ritualised behaviours, and a preference for predictability or sameness across routines and activities.

B3. Restricted or Highly Focused Interests

This criterion considers highly focused, intense, repetitive, or unusually restricted interests that may differ in intensity, focus, flexibility, or developmental appropriateness relative to peers.

B4. Sensory Differences and Sensory Regulation

This criterion considers sensory sensitivities, sensory avoidance, sensory-seeking behaviours, unusual sensory interests, altered sensory responses, and differences in sensory processing or sensory regulation across environments.

FUNCTIONAL IMPACT SUMMARY

CLINICAL FORMULATION AND CONSENSUS OPINION

RECOMMENDATIONS

LIMITATIONS

This report reflects clinical information available at the time of assessment. Findings should be interpreted within the broader developmental and clinical context of the client.

Kind Regards,



Vishal Maharaj

Registered Psychologist

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